

SCIENTIFIC REPORT · APS SUBGROUP · BERTONE ET AL. 2026

eCTM outperformed APS.

Head-to-head against the largest single hemoderivative class in the standard-of-care arm.

Background

OBJECTIVE

Autologous protein solution (APS) is the most widely used and best-studied hemoderivative for equine osteoarthritis. In a prior controlled trial, APS produced significant improvement over saline but the demonstrated benefit extended only to about 2 weeks; longer-term outcomes were unknown. This subgroup analysis compared a single dose of eCTM head-to-head with clinician-selected APS.

Methods

SUBGROUP

APS — 51% of all hemoderivatives used in this trial and the largest single product class. Compared with the full eCTM arm (n=76). Same lameness (AAEP), pain and swelling scales; same blinded scoring; same follow-up at Weeks 0, 2, 4, and 12.

ANALYSIS

Mixed-effects models with treatment × time as the fixed effect and horse as random. Cumulative improvement from baseline was compared at each visit (Supplementary Figure S3). This analysis was post-hoc, hypothesis-generating, and interpreted with caution given the smaller APS sample size.

WHY APS

APS was selected for the head-to-head because it is the largest single hemoderivative class in this trial and the one with the strongest prior efficacy signal in equine osteoarthritis. Beating APS is therefore the most demanding hemoderivative comparison available in these data.

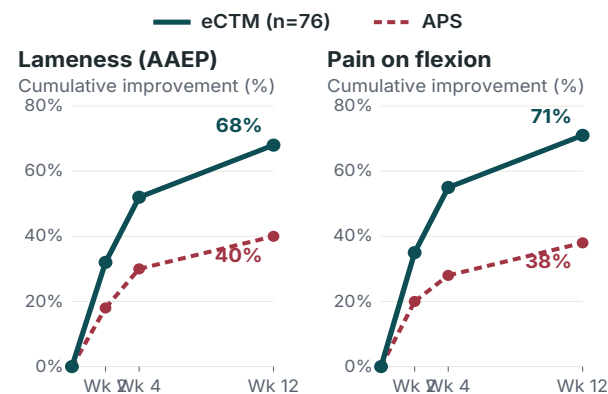
Results

eCTM vs. APS — CUMULATIVE IMPROVEMENT

Wk 2, 4 & 12

eCTM outperformed APS across multiple visits for both lameness and pain.

FIG 1 · eCTM vs. APS trajectories (Weeks 0–12)



Cumulative improvement from baseline (illustrative direction of Supplementary Figure S3). eCTM separated from APS by Week 2 and remained ahead for both lameness and pain.

Interpretation

BETTER THAN THE BEST HEMODERIVATIVE COMPARATOR

Against APS — the single hemoderivative with the strongest prior evidence in equine osteoarthritis — a single dose of eCTM produced greater cumulative improvement in lameness and pain across multiple visits over 12 weeks. Prior controlled evidence for APS extends to only about 2 weeks; the 12-week separation shown here supports a more durable biologic effect for eCTM.

Head-to-head vs. the reference hemoderivative.

eCTM outperformed APS across multiple timepoints in lameness and pain in a real-world practice-based cohort.

Equine Performance Labs